



Collected: _____
 Received: _____
 Case type: Surgical Case
 Physician: _____
 Accession: _____
 Sex: M

***** Amendment *****

Case has been amended

100-0-3
 Path: Carcinoma, papillary and follicular 8340/3
 Site: thyroid, NOS C73.9

DIAGNOSIS

(A) LEFT THYROID LOBE AND ISTHMUS

PAPILLARY THYROID CARCINOMA, FOLLICULAR VARIANT

Location: Left lobe

Multi-focal: No

Size = 4.5 cm

Extrathyroidal extension: Focally present, extending into fibroadipose tissue

Lymphovascular invasion: Absent

Resection Margins: Negative (tumor is less than 1 mm from cauterized edge)

Lymph nodes:

No lymph nodes present

Two benign intraglandular parathyroid glands.

(B) LEFT PARATRACHEAL NODE:

METASTATIC PAPILLARY THYROID CARCINOMA IN ONE LYMPH NODE (1/1).

No extranodal extension identified.

(C) RIGHT INFERIOR PARATHYROID GLAND:

Hyperplastic parathyroid tissue.

(D) RIGHT THYROID LOBE:

Thyroid tissue, no tumor present.

(E) BILATERAL CENTRAL NECK DISSECTION:

METASTATIC PAPILLARY THYROID CARCINOMA IN THREE OF SEVENTEEN LYMPH NODES (3/17).

No extranodal extension identified.

Benign thyroid tissue.

COMMENT

This case is amended to correct Specimen D in the diagnosis from "thigh" to "thyroid".

GROSS DESCRIPTION

(A) LEFT THYROID LOBE AND ISTHMUS – One pink-red thyroid lobe (5.5 x 5.0 x 2.9 cm) with a segment of isthmus (1.5 x 0.6 x 0.4 cm). The specimen is serially sectioned. One pink-red, well capsulated nodule (4.5 x 3.5 x 2.0 cm) is identified and

JOB:

Sex: M

Physician:

Accession:

Collected:

Pathologist:

Received:

Case type: Surgical Case

occupied most of the left lobe with a pink-red, homogenous cut surface with pale-gray capsule. No hemorrhage or necrosis identified. Representative section is submitted for frozen section evaluation. A portion of tumor is submitted for tumor bank. Specimen is representatively submitted.

SECTION CODE: A1, A2, representative section for frozen section evaluation; A3-A11, representative section of the tumor with capsule; A12, representative section of the isthmus.

*FS/DX: SUSPICIOUS FOR PAPILLARY THYROID CARCINOMA WITH FOLLICULAR VARIANT.

(B) LEFT PARATRACHEAL NODE – One pink-red lymph node (2.5 x 1.5 x 0.7 cm). The lymph node is bisected and entirely submitted in B for frozen section evaluation.

*FS/DX: CONSISTENT WITH METASTATIC CARCINOMA IN LYMPH NODE.

(C) ? RIGHT INFERIOR PARATHYROID GLAND – One pink-red tissue fragment (0.4 x 0.2 x 0.2 cm). Entirely submitted in C for frozen section evaluation.

*FS/DX: PARATHYROID TISSUE PRESENT.

(D) RIGHT THYROID LOBE – One pink-red thyroid lobe (5.0 x 3.0 x 1.2 cm) without orientation. The specimen is serially sectioned and the cut surface is pink-red without nodule. Representatively submitted in D1-D3.

(E) BILATERAL CENTRAL NECK DISSECTION – One pale-yellow fibroadipose tissue (4.5 x 4.0 x 1.2 cm) with multiple possible lymph nodes, ranging from 0.2 x 0.2 x 0.2 cm to 1.4 x 0.8 x 0.4 cm. The lymph nodes are entirely submitted.

SECTION CODE: E1-E5, each containing four possible lymph nodes; E6, two possible lymph nodes.

CLINICAL HISTORY

Suspicious for papillary thyroid carcinoma.

SNOMED CODES

T-B6000, T-C4200, M-80503, M-80506

"Some tests reported here may have been developed and performance characteristics determined by
These tests have not been specifically cleared or approved by the U.S. Food and Drug Administration."

Entire report and diagnosis completed by:

Page 2 of 3

Surgical Pathology Report

File under: Pathology

DOB:
Physician:

Sex: M

Collected:
Received:

Pathologist:
Case type: Surgical Case

Accession:

Entire report and diagnosis completed by:

Page 3 of 3	
Surgical Pathology Report	
File under: Pathology	
Criteria	Yes No
Diagnosis Discrepancy	✓
Primary Tumor Site Discrepancy	✓
HIPAA Discrepancy	✓
Prior Malignancy History	✓
Distal/Synchronous Primary Noted	✓
Case is (circle):	QUALIFIED / DISQUALIFIED
Reviewer Initials: RB	Date Reviewed: 7/27/12